PROJECT DOCUMENTATION

ClinicFlow Platform

Patient Booking, Records & Clinic Operations Platform

Project Planning & Management | Literature Review | Requirements Gathering

Course	[Course Name / Code]
Lecturer	[Lecturer Name]
Project Type	No-Code / Low-Code Client Delivery Project
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Project Team

Project Role	Team Member
UX & Discovery Lead	Ahmed Mohamed Abd-Elfatah
No-Code Database Lead	Rana Mohamed Hussein
No-Code Portal Lead	Abdalla Alaa-Elden Hassan
Automation & Workflow Lead	Kholoud Amr Mohamed
Testing & QA Lead	Aya Allah Mohamed
Integrations & Delivery Lead	Habiba Abdelmeneem

1. PROJECT PLANNING & MANAGEMENT

1.1 Project Proposal

1.1.1 Overview

Small medical clinics often face challenges in managing patient appointments, tracking medical histories, and handling high no-show rates due to manual scheduling systems. ClinicFlow is an integrated No-Code/Low-Code platform designed to streamline clinic operations by providing an intuitive online booking portal for patients, automated appointment reminders, and a central dashboard for clinic staff to efficiently manage daily schedules and secure medical records.

The project is delivered as a simulated client engagement. It begins with client discovery and UX planning, moves through a no-code build (database, forms, patient portal, dashboard, and reminder automations), and continues with portal building in Softr, workflow automation in Baserow, thorough testing and QA, and concludes with integrations and delivery.

1.1.2 Problem Statement

- Manual or phone-based scheduling leads to double-booked slots and scheduling conflicts.
- High no-show rates waste doctor time because patients are not reminded of their appointments.
- Patient records and visit history are scattered, making them hard to find and difficult to protect.
- Clinic management has no real-time view of appointment volumes, peak hours, or no-show statistics.

1.1.3 Project Objectives

- Provide an online portal where patients can browse services and doctors, view real-time slots, and book, cancel, or reschedule appointments.
- Eliminate double-booking through automatic slot validation and conflict prevention.
- Reduce the no-show rate by at least 30% through automated Email and SMS confirmations and reminders.
- Give receptionists a daily operational panel to manage walk-ins, manual bookings, and real-time visit statuses.
- Give doctors secure access to their schedules, availability settings, and visit notes.
- Give the Clinic Manager a live dashboard and role management through Role-Based Access Control (RBAC).
- Protect patient data with TLS/HTTPS in transit, AES-256 encryption at rest, hashed credentials, and daily backups.
- Deliver the solution with No-Code and Low-Code tools, together with documented automation flows and a client handover package.

1.1.4 Project Scope

In scope:

- Client discovery brief, personas, booking user flow, and wireframes.
- Structured database for patients, doctors, services, appointment slots, and appointments.
- Patient intake and booking forms, and a responsive patient-facing portal.
- Clinic operations dashboard, receptionist operations panel, and doctor portal with visit notes.
- Automated confirmations, reminders, and no-show handling via Email and SMS, with Google Calendar integration.
- Low-code scheduling engine, authentication for four roles (Patient, Doctor, Receptionist, Clinic Manager), and a backend records system.
- End-to-end automation chain (booking, confirmation, reminder, visit record, follow-up) and the final handover package.

Out of scope:

- Online payments, insurance claims, and billing.
- Telemedicine and video consultations.
- Advanced clinical records such as prescriptions, lab results, and medical imaging.
- Native mobile applications (the portal is a responsive web application).
- Live deployment with real patient data (testing uses synthetic data only).

1.1.5 Assumptions and Constraints

- The project is delivered by a team of six members over a 7-week period (16 October to 4 December 2026), aligned with the DEPI submission deadlines.
- The clinic is a simulated client, and all stakeholder input is gathered through simulated discovery sessions.
- Tools are limited to No-Code/Low-Code platforms available on free or low-cost plans.
- The platform design must be able to scale to additional doctors and branches without structural redesign.

1.2 Project Plan

The project is planned over 7 weeks, from 16 October 2026 to 4 December 2026, and organised into four delivery phases plus continuous project management. The four phases follow the DEPI deadlines: (A) Planning, Literature Review & Requirements (16 Oct), (B) System Analysis & Design (6 Nov), (C) Implementation (30 Nov), and (D) Testing, Reports & Final Presentation (4 Dec). Tasks within a phase run in parallel across the six team leads. The timeline below shows the planned duration of each task, followed by milestones, deliverables, and resource allocation.

1.2.1 Timeline (Gantt Chart)

Task	W1 16–22 Oct	W2 23–29 Oct	W3 30 Oct–5 Nov	W4 6–12 Nov	W5 13–19 Nov	W6 20–26 Nov	W7 27 Nov–4 Dec
A. Planning, Literature Review & Requirements (DEPI deadline: 16 Oct)							
Project planning & management	✔						
Literature review	✔						
Requirements gathering & client brief	✔						
B. System Analysis & Design (DEPI deadline: 6 Nov)							
Personas, user flow & wireframes		✔	✔				
Data model & system architecture design		✔	✔				
C. Implementation (DEPI deadline: 30 Nov)							
Database & intake/booking forms				✔	✔		
Scheduling engine & conflict prevention				✔	✔		
Authentication & role-based access				✔	✔		
Backend records system					✔	✔	
Patient booking portal					✔	✔	
Clinic dashboard					✔	✔	

Task	W1 16–22 Oct	W2 23–29 Oct	W3 30 Oct–5 Nov	W4 6–12 Nov	W5 13–19 Nov	W6 20–26 Nov	W7 27 Nov–4 Dec
Reminders & no-show automation					✔	✔	
Admin & operations panel						✔	✔
Calendar & SMS/Email integrations						✔	✔
End-to-end automation chain						✔	✔
D. Testing, Reports & Final Presentation (DEPI deadline: 4 Dec)							
Test plan & test cases				✔	✔		
Functional testing						✔	✔
Testing & user acceptance (UAT)							✔
Final report, handover & demo						✔	✔
E. Project Management							
Planning, risk & KPI review	✔	✔	✔	✔	✔	✔	✔
Milestones	**M1**		**M2**		**M3**	**M4**	**M5–M7**

Legend: shaded cells show the planned duration of each task by week (W1: 16–22 Oct, W2: 23–29 Oct, W3: 30 Oct–5 Nov, W4: 6–12 Nov, W5: 13–19 Nov, W6: 20–26 Nov, W7: 27 Nov–4 Dec); M1–M7 mark milestone completion weeks. DEPI deadlines: 16 Oct (Planning, Literature Review, Requirements), 6 Nov (System Analysis & Design), 30 Nov (Implementation), 4 Dec (Testing, Reports & Final Presentation).

1.2.2 Milestones

ID	Milestone	Completion Criteria	Target
M1	Planning, literature review & requirements submitted	Project plan, literature review, and requirements documentation completed and submitted.	16 Oct (Week 1)
M2	Analysis & design sign-off	Personas, booking user flow, wireframes, data model, and system architecture approved.	6 Nov (End of Week 3)
M3	Data model & forms ready	Database for patients, doctors, services, slots, and appointments built; intake and booking forms working with slot-availability logic.	19 Nov (End of Week 5)
M4	Portal, dashboard & automation working	Patients can browse, book, and view appointments; dashboard shows daily/weekly load; reminders and no-show flags run automatically.	26 Nov (End of Week 6)
M5	Implementation complete	Admin panel in use; role-based access, Google Calendar and SMS/Email connected; end-to-end chain working. Feature freeze.	30 Nov (Week 7)
M6	Testing & QA complete	Test plan executed, functional tests passed, UAT complete, security and performance verified against KPIs.	3 Dec (Week 7)
M7	Final presentation & reports delivered	Final report completed and final presentation and demo delivered.	4 Dec (Week 7)

1.2.3 Deliverables

ID	Deliverable	Responsible	Due
D0	Project planning, literature review, and requirements documentation (this document)	All team members	Week 1 (16 Oct)
D1	Analysis & design pack: discovery brief, personas, booking user flow, wireframes	UX & Discovery Lead	Week 3 (6 Nov)
D2	Structured database in Baserow (patients, doctors, services, slots, appointments)	No-Code Database Lead	Week 4 (12 Nov)
D3	Patient intake and appointment booking forms	UX & Discovery Lead	Week 5 (19 Nov)
D4	Patient-facing booking portal in Softr	No-Code Portal Lead	Week 6 (26 Nov)
D5	Clinic operations dashboard in Softr	No-Code Portal Lead	Week 6 (26 Nov)
D6	Automated reminders and no-show handling via Baserow automations	Automation & Workflow Lead	Week 6 (26 Nov)
D7	Test plan, test cases, and functional testing results	Testing & QA Lead	Week 7 (2 Dec)
D8	User acceptance testing (UAT) and security verification report	Testing & QA Lead	Week 7 (3 Dec)
D9	Internal admin and operations panel	Integrations & Delivery Lead	Week 7 (30 Nov)
D10	Documented automation flows (booking to follow-up)	Automation & Workflow Lead	Week 7 (30 Nov)
D11	Calendar and Email integrations (30 Nov) + final project report (4 Dec)	Integrations & Delivery Lead	Week 7 (30 Nov / 4 Dec)
D12	Live demo and final presentation	All team members	Week 7 (4 Dec)

1.2.4 Resource Allocation

Human resources: each team member leads one area and supports the others during peak phases.

Project Role	Team Member	Main Focus	Peak Phases
UX & Discovery Lead	Ahmed Mohamed Abd-Elfatah	**Client discovery, personas, booking user flow, wireframes, form UX design**	**A, B**
No-Code Database Lead	Rana Mohamed	**Data modeling in Baserow, structured tables for patients, doctors, services, slots, appointments**	**B, C**
No-Code Portal Lead	Abdalla Alaa-Elden Hassan	**Patient booking portal and clinic dashboard in Softr, connected to Baserow**	**B, C**
Automation & Workflow Lead	Kholoud Amr	**Baserow automations and webhooks for reminders, confirmations, and no-show handling**	**C**
Testing & QA Lead	Aya Allah Mohamed	**Test planning, test cases, functional testing, UAT, security and performance verification**	**C, D**
Integrations & Delivery Lead	Habiba Abdelmeneem	**Admin panel, Google Calendar and Email integrations, final automation sprint, delivery packaging**	**C, D**

Tools and technology (proposed stack; the final choice and rationale are documented in the final report):

Category	Proposed Tool	Purpose
UX design	**Figma**	**Personas, user flows, and wireframes**
No-code database & forms	**Baserow**	**Data model, tables, intake and booking forms, automations**
No-code portal & dashboard	**Softr**	**Patient-facing booking portal, clinic dashboard, and admin views**
Automation	**Baserow Automations + Webhooks**	**Confirmations, reminders, and no-show handling**
Integrations	**Google Calendar, Email (SMTP)**	**Calendar sync and patient notifications**
Project management	**Trello / Notion, Google Drive**	**Task tracking, documentation, and version control of files**

1.3 Task Assignment & Roles

Each team member holds one lead role with clearly defined responsibilities. All members contribute to testing, documentation, and the final demo.

1.3.1 Roles and Responsibilities

Role	Responsibilities	Key Deliverables
UX & Discovery Lead	**Conduct client discovery and write the discovery brief. Create personas, booking user flow, and wireframes for patient and staff experiences. Design the no-code forms and verify the user experience.**	**D1, D3**
No-Code Database Lead	**Design and build the structured data model in Baserow. Create and configure all database tables (patients, doctors, services, slots, appointments). Build intake and booking forms.**	**D2**
No-Code Portal Lead	**Build the patient-facing booking portal in Softr. Build the clinic operations dashboard and doctor portal views. Connect Softr pages to Baserow data.**	**D4, D5**
Automation & Workflow Lead	**Configure Baserow automations for booking confirmations and reminders. Set up no-show detection and automated follow-up workflows. Document all automation flows.**	**D6, D10**
Testing & QA Lead	**Create the test plan and test cases for all features. Run functional testing, user acceptance testing (UAT), and security verification. Verify KPIs and non-functional requirements.**	**D7, D8**
Integrations & Delivery Lead	**Build the internal admin and operations panel. Integrate Google Calendar and Email notification services. Package the handover and final report.**	**D9, D11**
All team members	**Test the system, write documentation, and present the live demo.**	**D0, D12**

1.3.2 Responsibility Assignment Matrix (RACI)

R = Responsible (does the work), A = Accountable (owns the result), C = Consulted, I = Informed.

Task	UX & Discovery	No-Code DB	No-Code Portal	Automation	Testing & QA	Integrations
Client discovery brief	R/A	C	I	I	I	I
Personas, user flow & wireframes	R/A	C	C	I	I	I
Data model & database	C	R/A	C	I	I	I
Intake & booking forms	R/A	C	I	I	I	I
Patient booking portal	C	C	R/A	I	I	I
Clinic dashboard	C	C	R/A	I	I	C

Reminders & no-show automation	I	C	I	R/A	C	C
Booking confirmation workflow	I	C	I	R/A	C	I
Admin & operations panel	C	I	C	I	I	R/A
Calendar & Email integrations	I	C	I	C	C	R/A
End-to-end automation chain	I	C	C	R/A	C	R/A
Test plan & test cases	C	C	C	C	R/A	C
Functional & user acceptance testing (UAT)	R	R	R	R	R/A	R
Security & performance verification	I	C	C	C	R/A	C
Final report & documentation	R	R	R	R	R	R/A
Demo & handover presentation	R	R	R	R	R	R/A

1.4 Risks Assessment & Mitigation Plan

Each risk is scored as Likelihood (L) × Impact (I), both on a scale of 1 to 5. A score of 15 or more is rated High, 8 to 14 Medium, and 7 or less Low. The team leads review the register weekly, and any High risk is escalated to the full team immediately.

ID	Risk	L	I	Score	Rating	Mitigation	Owner
R1	Unauthorized access to patient data, including wrong role permissions	3	5	15	**High**	AES-256 at rest and TLS in transit; RBAC tested with one test account per role; hashed passwords; session timeout.	Testing & QA Lead
R2	Double-booking caused by simultaneous bookings	4	4	16	**High**	Server-side slot validation with a unique slot constraint; test concurrent bookings before release.	Automation & Workflow Lead
R3	No-code/Softr + Baserow platform limits (speed, record limits) cannot support 100 concurrent users	3	4	12	Medium	Test the tools early in Week 1; run a load test with 100 users; move complex logic to the low-code layer.	No-Code Database Lead

ID	Risk	L	I	Score	Rating	Mitigation	Owner
R4	Email/SMS or calendar integration fails and reminders are missed	3	4	12	Medium	Automatic retries; Email fallback if SMS fails; alerts on failed automation runs.	Integrations & Delivery Lead
R5	Data loss during no-code to low-code migration or during an outage	2	5	10	Medium	Daily automatic backups; backup before every migration; test one restore.	No-Code Database Lead
R6	Low adoption because staff or patients find the system hard to use	3	4	12	Medium	Test wireframes with real users; main tasks in 3 clicks or fewer; mobile-first design; short staff guide.	UX & Discovery Lead
R7	Scope creep or schedule delays between project phases	4	3	12	Medium	Freeze requirements in Section 3.3; keep a change-request log; weekly milestones and check-ins.	UX & Discovery Lead
R8	Insufficient testing before handover	3	4	12	Medium	Test plan covering the full booking-to-follow-up chain; user acceptance test; feature freeze on 30 Nov, four days before the 4 Dec final presentation.	Testing & QA Lead

1.5 KPIs (Key Performance Indicators)

The KPIs below measure whether ClinicFlow meets the non-functional requirements in Section 3.4 and achieves the project objectives. Targets are to be confirmed with the client, and baseline values (such as the current no-show rate) are captured during discovery.

ID	KPI	Target	Measured By	Frequency
K1	**Response time**	≤ 2 seconds for page loads and slot-availability checks	Uptime / performance tool	Weekly
K2	**System uptime**	≥ 99.5% during clinic operating hours	Uptime monitor	Monthly
K3	**Concurrent users**	100 users with no slowdown	Load test	Before launch
K4	**Double-booking incidents**	0	Database conflict log	Weekly
K5	**User adoption rate**	Staff ≥ 90% active; ≥ 60% of bookings made online by month 3	Login and booking-source data	Monthly
K6	**Booking completion rate**	≥ 80% of started bookings completed	Portal funnel tracking	Bi-weekly
K7	**No-show rate**	≥ 30% reduction vs. baseline	Dashboard status data (Absent)	Monthly
K8	**Reminder delivery rate**	≥ 98% of reminders delivered	Email / SMS service logs	Weekly
K9	**Staff task efficiency**	Primary tasks (e.g., check-in) in ≤ 3 clicks	Usability test	Per test round

ID	KPI	Target	Measured By	Frequency
K10	**Security incidents**	0 unauthorized access cases; 100% of role tests passed	Audit logs, test results	Before each release

2. LITERATURE REVIEW

2.1 Feedback & Evaluation

This section records the lecturer's assessment of the ClinicFlow project. The team's preliminary self-evaluation is provided first, followed by the lecturer's feedback form.

2.1.1 Team Self-Evaluation (Preliminary)

Strengths:

- Requirements are clear, structured by stakeholder, and measurable (for example, 2-second response time and 99.5% uptime).
- The design addresses the core clinic problems: double-booking, no-shows, and scattered records.
- Security is built in from the start through RBAC, encryption, and daily backups.
- The staged approach (no-code first, then low-code) reduces delivery risk and documents tool trade-offs.

Limitations:

- Discovery is simulated, so requirements have not been validated by a real clinic.
- Free-tier limits of the chosen platforms may affect load capacity and integrations.
- Testing uses synthetic data, so real clinical workloads are not yet verified.

2.1.2 Lecturer's Feedback Form

Assessment Area	Lecturer's Comments
Project Planning & Management	
Literature Review	
Requirements Gathering	
Implementation	
Testing	
Presentation	
Overall Remarks	

Evaluation Details	
Evaluated by	
Date	
Signature	

2.2 Suggested Improvements

The following enhancements would extend ClinicFlow beyond the current scope. They are ranked by timeframe so the client can plan future phases. Improvements suggested by the lecturer can be added to this list after evaluation.

#	Improvement	Description and Expected Benefit	Related Area	Timeframe
1	**WhatsApp reminders**	Send confirmations and reminders over WhatsApp in addition to Email and SMS to improve delivery and response rates.	Notifications	Short term
2	**Arabic / English interface**	Bilingual portal with right-to-left support so more patients can use it comfortably.	Usability	Short term
3	**Waitlist and auto-fill of slots**	Offer cancelled slots to waiting patients automatically to increase doctor utilization.	Scheduling	Short term
4	**Two-factor authentication**	Add a second login step for doctors, receptionists, and managers to strengthen data protection.	Security	Short term
5	**Online payments and deposits**	Collect booking deposits or fees online to reduce no-shows and speed up billing.	Business	Medium term
6	**Patient feedback surveys**	Send a short satisfaction survey after each visit as part of the follow-up automation.	Quality	Medium term
7	**Monitoring and audit dashboard**	Track system errors, uptime, and record-access logs in one place for administrators.	Reliability	Medium term
8	**Patient health portal**	Let patients view visit summaries, prescriptions, and lab results securely.	Records	Long term
9	**Multi-branch support**	Extend the data model and dashboard to manage several clinic branches.	Scalability	Long term
10	**Predictive no-show analytics**	Use booking history to flag high-risk appointments and target extra reminders.	Analytics	Long term
11	**TelemedicineR**	Offer video consultations for follow-up visits.	Services	Long term

2.3 Final Grading Criteria

3. REQUIREMENTS GATHERING

3.1 Stakeholder Analysis

The platform serves four stakeholder groups. Each has distinct needs and uses a dedicated interface, and access is controlled through Role-Based Access Control.

Stakeholder	Needs and Responsibilities	Main Interface
Patients	Search for clinic services, view available time slots, book/cancel appointments online, and receive automated notifications.	Patient portal
Doctors	Manage their working schedules, view daily appointment lists, and update basic patient visit notes and records.	Doctor portal
Receptionists	Manage daily clinic check-ins, oversee overall schedules, handle manual bookings/rescheduling, and follow up with patients.	Operations panel
Clinic Manager	Oversee whole platform configurations, track operational dashboards, manage user roles, and monitor performance.	Admin panel and dashboard

3.2 User Stories & Use Cases

3.2.1 User Stories

ID	As a...	I want to...	So that...
US1	Patient	browse available doctor time slots and book an appointment online	I can reserve my visit easily without waiting on the phone.
US2	Patient	receive automated appointment reminders via email/SMS	I do not miss my scheduled visit.
US3	Receptionist	have an operational panel to view all today's appointments and update their status (Checked-in, Completed, and Absent)	I can manage the clinic flow smoothly.
US4	Doctor	view my daily patient list and access/update visit notes	I can keep an accurate medical record for each patient.
US5	Clinic Manager	see a central dashboard showing appointment volumes and no-show statistics	I can monitor and optimize clinic operations.

3.2.2 Use Case: Patient Appointment Booking

Use Case ID	UC-01
Use Case Name	Patient Appointment Booking
Actor	Patient
Preconditions	Services, doctors, and working hours are configured; the patient has internet access.

Main Flow	1. The patient visits the ClinicFlow portal. 2. The patient selects the required service or doctor. 3. The system displays available dates and time slots. 4. The patient selects a slot and fills in contact details. 5. The system validates slot availability (prevents double booking). 6. The system confirms the booking and sends a confirmation notification.
Alternative Flows	4a. Contact details are invalid: the system highlights the errors and asks the patient to correct them. 5a. The slot has just been taken: the system informs the patient and shows other available slots.
Post-conditions	Appointment is saved in the database and visible on the clinic dashboard.

3.3 Functional Requirements

Priorities follow the MoSCoW method: Must have (essential for the first release) and Should have (important but not critical).

ID	Requirement	Description	Priority
A. Patient Management & Online Booking			
FR-01	**Service & Schedule Browsing**	The system shall allow patients to search for medical services, view doctor profiles, and check available real-time time slots.	Must
FR-02	**Online Appointment Booking**	Patients shall be able to select a date/time slot and submit their contact details to finalize an appointment reservation.	Must
FR-03	**Slot Validation & Conflict Prevention**	The platform shall automatically validate time slots to prevent double-booking or scheduling conflicts.	Must
FR-04	**Cancellation & Rescheduling**	Patients shall have the option to reschedule or cancel their appointments prior to the clinic's designated policy cutoff.	Should
FR-05	**Automated Notifications**	The system shall automatically send appointment confirmations and reminders to patients via Email and SMS.	Must
B. Receptionist Operations & Flow Control			
FR-06	**Daily Schedule Panel**	Receptionists shall have an operational view displaying all appointments scheduled for the current day.	Must
FR-07	**Status Updates**	Receptionists shall be able to update patient visit statuses in real time (e.g., Checked-in, In Progress, Completed, Absent/No-Show).	Must
FR-08	**Manual Booking & Overrides**	Receptionists shall be able to manually create, edit, or adjust appointments for walk-in patients or phone reservations.	Must
C. Doctor Portal & Visit Records			
FR-09	**Personal Schedule View**	Doctors shall be able to view their daily and weekly appointment rosters.	Must

ID	Requirement	Description	Priority
FR-10	**Visit Notes Management**	Doctors shall be able to create, view, and update essential patient visit notes and basic clinical details for each appointment.	Must
FR-11	**Availability Management**	Doctors shall be able to set and modify their available working hours and mark planned time off.	Should
D. Administration & Platform Configuration			
FR-12	**Role-Based Access Control (RBAC)**	The system shall support secure user authentication and enforce role-based permissions for Patients, Doctors, Receptionists, and Clinic Managers.	Must
FR-13	**Operational Dashboard & Analytics**	The platform shall generate real-time metrics and reports on appointment volumes, peak hours, and overall no-show rates.	Must

3.4 Non-Functional Requirements

ID	Requirement	Description
1. Security & Data Protection		
NFR-01	**Data Encryption**	All sensitive patient data and medical notes must be encrypted both in transit (using TLS/HTTPS) and at rest (using AES-256 encryption).
NFR-02	**Access Control**	User passwords and access tokens must be hashed, and session timeouts should be enforced after inactivity.
2. Performance & Responsiveness		
NFR-03	**Response Time**	Page loads and key API requests (such as slot availability checks) must complete within 2 seconds under normal traffic conditions.
NFR-04	**Concurrent Users**	The system must handle up to 100 concurrent users without degradation in response time.
3. Availability & Reliability		
NFR-05	**System Uptime**	The platform shall maintain an operational availability of 99.5% during clinic operating hours.
NFR-06	**Data Backup**	Automatic database backups must run on a daily schedule to prevent data loss.
4. Usability & Accessibility		
NFR-07	**User Interface**	The online portal must feature a clean, responsive layout optimized for mobile devices, tablets, and desktop browsers.
NFR-08	**Ease of Use**	Clinic staff should be able to perform primary tasks (such as checking in a patient) in 3 clicks or fewer.
5. Scalability & Low-Code Maintainability		
NFR-09	**Architecture**	The platform architecture must support scaling to accommodate additional doctors, branches, or higher booking volumes without structural redesign.

ID	Requirement	Description
NFR-10	**Low-Code/No-Code Agility**	System workflows, automated emails, and dashboards should be configured using standard Low-Code/No-Code components for ease of maintenance.